

Inpatient Hematology-Oncology Service - Progress Note

DATE: November 1, 2023 (Hospital Day 4)

Patient: COOPER, Carl (MRN: AML065, DOB: 01-Jun-1981)

SUBJECTIVE:

Patient is a 42-year-old male with favorable-risk AML dx on Sept 25, admitted for his first cycle of consolidation chemotherapy. This is Consolidation Cycle #1, Day +4. He reports feeling "okay, a little tired." He denies nausea or vomiting, currently taking scheduled Zofran. He complains of mild eye irritation and grittiness, despite using his prescribed eye drops. No headache, dizziness, or changes in balance. Tolerating a regular diet. Moving bowels.

OBJECTIVE:

Vitals: T 37.1°C, HR 72, BP 120/78, RR 16, SpO2 98% on RA.

General: Well-appearing male, alert and oriented x4.

HEENT: Conjunctiva are injected bilaterally, no significant chemosis. Oral mucosa is moist and pink, no mucositis.

CV: RRR, no m/r/g.

Lungs: CTAB.

Neuro: Cranial nerves II-XII intact. Finger-to-nose and heel-to-shin tests are performed without dysmetria. Gait is steady. No nystagmus. (Formal cerebellar exam normal).

Skin: No new rashes.

Labs (0600):

- WBC 2.1 K/uL (down from 7.8 on admission)
- ANC 1.1 K/uL
- Hgb 9.8 g/dL
- Plt 112 K/uL (down from 250 on admission)
- **CMP:** K 3.6, Mg 1.8. Otherwise WNL. Cr 0.9.

ASSESSMENT & PLAN:

*Mr. Cooper is a 42-year-old male with favorable-risk AML, t(8;21)(q22;q22.1), status-post 7+3 induction chemotherapy. His Day +28 bone marrow biopsy on October 30 confirmed a complete remission. Molecular testing for the common **KIT mutation** was negative. He is now receiving his first of four planned cycles of consolidation with **High-Dose Cytarabine (HiDAC)** at a dose of 3g/m² every 12 hours on Days 1, 3, and 5.*

1. **AML in Remission / Consolidation Therapy:** *He has successfully completed all doses of HiDAC for this cycle (final dose was yesterday evening). He is entering the expected myelosuppressive phase.*
 - **Plan:** Continue daily CBC monitoring. Anticipate nadir around Day +7-10. Hold transfusion parameters at Hgb < 7.0 and Plt < 10.0.
2. **Chemotherapy Side Effect Management:**
 - **Chemical Conjunctivitis:** *His eye irritation is a classic side effect of HiDAC.*
 - **Plan:** Increase frequency of Prednisolone Acetate 1% eye drops to every 4 hours while awake for the next 48 hours. Stress importance of adherence.
 - **Cerebellar Toxicity:** *He remains at risk for this for several more days.*

- **Plan:** *Continue formal neurologic checks with nursing every shift. Continue to hold his morning dose of chemotherapy (as he is done) and assess his handwriting and gait.*

3. **Supportive Care:**

- **Prophylaxis:** *Continue on Levaquin, Valacyclovir, and Micafungin.*
- **Electrolytes:** *Repleted Potassium and Magnesium intravenously this morning.*
- **Nausea:** *Continue scheduled Ondansetron. Add Prochlorperazine PRN.*

4. **Disposition:** *He remains afebrile and clinically stable. If he remains so, he is on track for discharge in 5-7 days once his counts enter the nadir and we confirm he can be safely managed as an outpatient.*